PROJECT AID

Expert Instructions

Healthcare Multimodal Triage — Image, Domain & Context Note

Phase 1 instruction set. In this phase you submit three things per task: the domain, the image, and a short context note. The prompt, target answer, expert reasoning, and scoring rubric are produced separately in a later step.

1. Overview

Welcome

Thanks for joining Project Aid. You will be helping build a high-quality healthcare visual-reasoning dataset. Each task you create pairs a single, real-world clinical photograph with one of five clinical domains and a short note naming what the image shows. The dataset is used to test and train models on a very specific skill: looking at one photo and making a safe, well-reasoned decision about what a patient should do next.

This is the same kind of decision a triage nurse makes — identify what is going on in the image, then decide whether the situation can be managed at home, needs a routine or urgent appointment, or needs to be escalated to emergency evaluation. Your job is to supply the clear, well-chosen images that make that decision possible.

What you will be doing

For each task in this phase you will do exactly three things:

1. Find a real, publicly available clinical image that fits one of the five project domains.
2. Select the correct domain for that image from the fixed list of five.
3. **Add a short context note** — a few words naming what the image shows (for example, “This is melanoma”).

That is the complete submission for this phase: domain, image, and context note. You do not write the prompt, the answer, the reasoning, or the rubric. The triage prompt, the target answer, the expert reasoning, and the pass/fail scoring rubric will be made by another team using your image and context note as the starting point.

> **Why your image still has to be excellent:** Because the prompt, answer, and rubric are all built from the image you submit, a weak image produces a weak datapoint. A good image shows the clinically relevant features clearly enough that a nurse can build several distinct, checkable claims from it — an identification, a triage decision, supporting evidence, and the red-flags that would change the decision. Pick images a nurse could build four or five scoring points from.

The shape of a complete datapoint (for context)

So you understand what your submission feeds into, here is what a finished datapoint looks like. You are responsible only for the first three rows.

Image	The clinical photograph (you provide this).
Domain	One of the five fixed domains (you provide this).
Image context	A few words naming what the image shows, e.g. “This is melanoma” (you provide this).
Prompt	The triage question the model is asked about the image.
Target answer	The correct answer as numbered key facts — identification plus the assist-vs-escalate decision.
Expert reasoning	A numbered, evidence-grounded walkthrough of how a clinician reaches each fact.
Rubric + disqualifier	Numbered pass/fail criteria, plus a hard safety “Disqualifier” that auto-fails any unsafe triage call.

2. The Five Domains

Every task belongs to exactly one of the five domains below. You will select it on the submission form. These five are the complete list — do not invent new domains, and do not submit images that do not clearly belong to one of them. The domains were chosen with practicing-nurse input as cases that are clinically common but that current AI models still mislabel or mis-rank in urgency, so this is exactly where good data helps most.

Choosing your domain first is the easiest way to stay focused: it tells you what kind of image to hunt for and what the triage decision will eventually turn on.

Domain	What it covers	The triage question it covers
Wound Triage	Cuts, abrasions, lacerations, ulcers, pressure sores, and other open or healing skin wounds.	Home wound care vs. medical evaluation (routine, urgent, or same-day).
Skin Lesion Screening	Moles, pigmented spots, and other lesions assessed for features of skin cancer.	Routine self-monitoring vs. dermatology referral / biopsy.
Burn Classification	Thermal, scald, sun, chemical, or contact burns at any depth.	Home care vs. urgent care, ED, or burn-center referral.
Rash Identification	Rashes and skin eruptions identified by their pattern and body distribution.	Self-limited / OTC care vs. prescription treatment, plus infection-control needs.
Red-Eye Triage	A red, irritated, or bloodshot eye and the surrounding lids.	Reassurance vs. routine/urgent care vs. same-day emergency (vision-threatening).

Wound Triage

Open or healing skin wounds — cuts, scrapes and abrasions, lacerations, puncture wounds, surgical incisions, diabetic and pressure ulcers, venous leg ulcers, and bites. The triage question is whether the wound can be managed with ordinary home wound care or whether it needs medical evaluation, and if so, how soon.

Good images to look for: close-ups where the wound bed and the skin around it are clearly visible — a fresh laceration with gaping edges, an abrasion forming a healthy scab, a chronic ulcer on the lower leg or foot with surrounding discoloration, a pressure sore, or a wound showing early infection (spreading redness, swelling, drainage). Try to collect both clearly minor wounds and clearly serious ones so the dataset stays balanced across dispositions.

What makes a strong example: the visible features should be readable enough to drive a decision — wound depth, edges, the wound bed, surrounding skin color, and signs of infection or chronicity should all be assessable from the photo.

Skin Lesion Screening

Moles and other pigmented spots are evaluated for features of skin cancer, most often using the ABCDE framework (Asymmetry, Border, Color, Diameter, Evolution). The triage question is whether the lesion looks reassuring enough for routine self-monitoring or concerning enough to warrant dermatology referral and possible biopsy.

Good images to look for: single, well-lit, in-focus close-ups of one lesion. Collect both reassuring lesions (symmetric, even border, single uniform color) and concerning ones (asymmetric, notched or blurred borders, multiple colors such as black, brown, tan, red, or blue). Dermatology atlases and public-domain medical image libraries are good sources.

What makes a strong example: the lesion fills enough of the frame that asymmetry, border, and color are all assessable. Note that diameter and evolution usually cannot be judged from a single static photo — an image that makes that limitation obvious is still valuable.

Burn Classification

Burns from heat, scalding liquids, sun, chemicals, friction, or contact, assessed by depth (superficial / superficial partial-thickness / deep partial-thickness / full-thickness) and by how much body surface area is involved. The triage question is whether the burn is suitable for home care or needs urgent care, an emergency department, or a specialized burn center.

Good images to look for: burns where depth cues are visible — redness without blistering (superficial), blisters and a moist pink wound bed (partial-thickness), or dry, leathery, white or charred tissue (full-thickness). Images that show a burn over a high-risk area (hands, face, feet, major joints, or genitals) are especially valuable because location can change the level of care regardless of depth, under American Burn Association referral criteria.

What makes a strong example: the photo lets a viewer estimate both depth and rough extent, and ideally shows a feature (a blister, the involved body part, the size of the area) that the triage decision can hinge on.

Rash Identification

Rashes and skin eruptions identified from their morphology (what the individual spots look like) and their distribution (where on the body and in what pattern). The triage question covers whether the condition is self-limited or OTC-treatable versus needing prescription treatment, and what infection-control or environmental precautions apply.

Good images to look for: rashes with a recognizable pattern — linear streaks of blisters (plant contact dermatitis), spots concentrated on the palms or soles (hand-foot-and-mouth), hives, a spreading bullseye, or a rash following a band on one side of the body. The location and arrangement should be visible, since distribution is half the diagnosis.

What makes a strong example: the individual lesions and the overall layout are both clear enough to reason about, so morphology and distribution can be used together and any contagiousness or precautions can be flagged.

Red-Eye Triage

A red, bloodshot, irritated, or discharging eye, together with the lids and surrounding area. The triage question separates benign, reassurance-only findings from those needing routine or urgent care, and from the true vision-threatening emergencies.

Good images to look for: clear close-ups of the eye where the pattern of redness, any discharge, the cornea, and the pupil are visible. Useful presentations include diffuse redness with thick yellow discharge crusting the lashes (bacterial conjunctivitis), a sharply bordered patch of blood on the white of the eye (subconjunctival hemorrhage), watery red eyes (viral or allergic), or red-flag signs such as a hazy cornea or a ring of deep redness around the iris.

What makes a strong example: enough of the eye is in focus that discharge type, redness pattern, corneal clarity, and pupil shape can be judged — these are the very features the triage decision turns on.

> **Balance your submissions:** Across all domains, deliberately mix clearly benign cases (home care, reassurance, routine monitoring) with clearly serious ones (urgent care, ED, biopsy, burn center). A dataset that is all-emergencies or all-trivial teaches a model very little. If you have submitted three “send to the ED” wounds in a row, go find a healing scrape next.

3. Finding the Image

Each task is one image. The image is the whole basis for the datapoint, so its quality is the most important thing you control.

Image requirements

- **Real, unaltered photographs only.** Every image must be a genuine photograph of a real person or body part. No AI-generated images, no illustrations, diagrams, or drawings, and no photos that have been edited or photoshopped.
- **Clear and reasonably high quality.** The clinically relevant features — the wound bed, the lesion's borders, the burn's texture, the rash pattern, the eye's discharge — must be visible enough to reason about. Blurry, dark, tiny, or heavily compressed images are not acceptable.
- **One image per task.** A single photo, not a collage, a side-by-side, or a multi-image grid.
- **Shows a clinically relevant feature.** The feature that defines the case — the discharge, the border, the burn depth — has to actually be in the frame. Do not submit an image where the key finding is cropped out or not visible.

No duplicate or similar images

Every image you submit must be unique. Duplicate or near-duplicate images add no new information to the dataset and will be rejected.

- **No exact duplicates.** Do not submit the same image more than once, and do not submit an image that has already been submitted — by you or anyone else — on the project.
- **No near-duplicates or minor variants.** Do not re-submit an image you have already used after cropping, rotating, resizing, recoloring, or otherwise lightly editing it. These count as the same image.
- **No multiple shots of the same case.** Several photos of the same wound, lesion, or eye — even from slightly different angles, distances, or lighting — count as similar images. Submit only one, and look for a genuinely different case for your next task.
- **No visually similar cases.** Avoid stacking your submissions with images that look alike or show the same presentation. The dataset gains from variety across body sites, skin tones, severities, and presentations — not repetition.

> **Rule of thumb:** If a reviewer placed your new image next to one already in the dataset and could not quickly tell them apart, it is too similar and will be rejected. When in doubt, find a fresh image.

Copyright and watermarks

You may use any image you find online, as long as the image itself carries no copyright notice and no watermark printed on it. The practical rules:

- **No watermarks on the image.** Do not use a photo with a stock-agency watermark, a website name, a logo, or any text stamped across it. If there is a watermark, find a different image.
- **No visible copyright notice on the image.** If the photo itself displays a copyright line or "©" credit baked into the picture, skip it.

- **Screenshots are allowed.** You may take a screenshot of an image you find online and submit that screenshot. Crop the screenshot down to just the relevant photo so there are no surrounding page elements, captions, watermarks, or logos in the frame.

When you crop or screenshot, make sure you are not cropping out a clinically relevant feature, and that the result is still sharp and clear.

Where to look

Good starting points for finding clinically useful, public images:

- **Public medical and educational image libraries —** dermatology atlases, nursing and first-aid education sites, and open medical-image collections are excellent for lesions, rashes, burns, and eye findings.
- **Government and health-agency sites —** CDC, NIH, and similar agencies publish clinical photos, many in the public domain.
- **Wikimedia Commons and Creative Commons searches —** large libraries of openly licensed medical photographs.
- **Reddit and community forums —** medical-advice and first-aid communities are full of real-world, non-staged photos of wounds, rashes, burns, and eyes. Real conditions in real lighting are exactly what this dataset wants.

Wherever you find an image, save the source URL somewhere in case a reviewer asks where it came from.

What to avoid

- **Personally Identifiable Information.** Avoid or crop images showing anything that could identify the person in the image (full face, biometric data, hospital wristbands (name, DOB, address), medical records, ID badges, distinctive tattoos, license plates, or room labels.
- **Watermarks, logos, or text overlays.** Anything stamped on the image, especially text that gives away the answer (a caption naming the condition).
- **Duplicate or similar images.** Anything that repeats — or closely resembles — an image already in the dataset (see “No duplicate or similar images” above).
- **Staged, too-perfect stock photos.** Clean studio shots often do not reflect real-world presentations and make poor triage cases.
- **Ambiguous or off-topic images.** If you cannot clearly tell what is going on, neither can the model, and neither can the nurse writing the answer. Pick something readable.
- **Edited or AI-generated images.** If it looks photoshopped, smoothed, or synthetic, do not use it.

4. The Image Context Note

Along with the image and the domain, every submission includes one short context note: a few words — one short sentence at most — that plainly name what the image shows. If the photo is of a melanoma, you write “This is melanoma.” If it is a healing scrape, you write “This is a healing abrasion.” That is the whole field.

It exists so there is a quick, human-readable label of each image’s subject — it lets images be organized, balanced across dispositions, and sanity-checked at a glance. The context note is internal metadata: the model being evaluated never sees it (the model receives only the image and the prompt), so it does not leak the answer.

How to write it

- **Keep it short.** A few words, one short sentence maximum. “This is bacterial conjunctivitis.” — not a paragraph. If you are writing more than one sentence, it is too long.
- **Name what the image is, not what to do about it.** The context note is the subject label only. It is not the triage decision, the reasoning, or the rubric — those come in later steps. “This is a partial-thickness burn on the palm” is right; “This is a burn that needs a burn center” adds the disposition and is not what this field is for.
- **Make it accurate.** If you are not certain what the image shows, research it until you are, or choose a clearer image. A wrong context note misleads the team that builds the answer.

Examples by domain

Each of these is the right length — a few words naming the subject. Aim for this level of brevity.

Domain	Example context notes
Wound Triage	*“This is a diabetic foot ulcer.”* • *“This is a healing abrasion.”*
Skin Lesion Screening	*“This is melanoma.”* • *“This is a benign mole.”*
Burn Classification	*“This is a partial-thickness burn on the palm.”* • *“This is a sunburn.”*
Rash Identification	*“This is hand-foot-and-mouth disease.”* • *“This is poison ivy.”*
Red-Eye Triage	*“This is bacterial conjunctivitis.”* • *“This is a subconjunctival hemorrhage.”*

> **Short means short:** The context note is the one place in this project where less is more. A few words is the target. Save every other observation — severity, the reason it is what it is, what the patient should do — for later steps.

5. Worked Examples — Two per Domain

Below are two approved examples for each domain — a more serious case paired with a milder one — showing the things you submit: the image, the domain, and a short context note. Study how the context note stays to a few words and names the subject accurately, and how the two cases in each domain land on different presentations. That contrast is exactly the balance you should aim for across your own submissions.

Example W1 — Diabetic foot ulcer — Wound Triage



A wound on the weight-bearing sole of the foot, surrounded by thick callus.

Domain Wound Triage

Image context *This is a diabetic neuropathic foot ulcer.*

Example W2 — Healing abrasion — Wound Triage



A scraped area of skin with reddish scabbing and no open, gaping edges.

Domain Wound Triage

Image context *This is a superficial abrasion.*

Example L1 — Irregular pigmented lesion — Skin Lesion Screening



A single dark lesion with an irregular shape and uneven color.

Domain Skin Lesion Screening

Image context *This is melanoma.*

Example L2 — Uniform mole — Skin Lesion Screening



A single small, symmetric, evenly brown lesion with a smooth border.

Domain Skin Lesion Screening

Image context *This is a benign mole.*

Example B1 — Burn on the palm — Burn Classification



A burn on the palm of the hand showing blistering and a moist wound bed.

Domain Burn Classification

Image context *This is a partial-thickness burn on the palm.*

Example B2 — Sunburn on the back — Burn Classification



Widespread red skin across the upper back with no blistering visible.

Domain Burn Classification

Image context *This is a sunburn (superficial burn).*

Example R1 — Rash on the palm — Rash Identification



Multiple small red spots scattered across the palm and fingers.

Domain Rash Identification

Image context *This is hand-foot-and-mouth disease.*

Example R2 — Linear blistering rash — Rash Identification

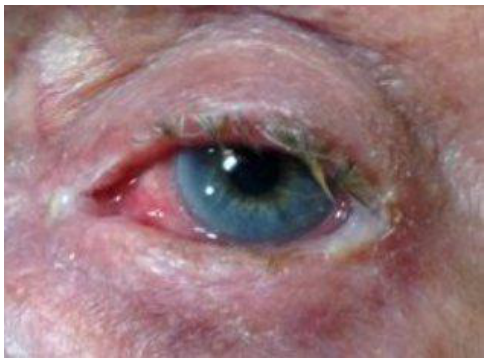


Streaky lines of small blisters and redness running across the skin.

Domain Rash Identification

Image context *This is poison ivy (allergic contact dermatitis).*

Example E1 — Red eye with discharge — Red-Eye Triage



A diffusely red eye with thick discharge crusting the lashes and mild lid swelling.

Domain Red-Eye Triage

Image context *This is bacterial conjunctivitis.*

Example E2 — Red patch on the eye — Red-Eye Triage



A sharply bordered patch of blood on the white of the eye, with a clear cornea and normal pupil.

Domain Red-Eye Triage

Image context *This is a subconjunctival hemorrhage.*

6. Before You Submit

Run through this checklist on every task. Remember: in this phase you are submitting the domain, the image, and a short context note.

- The image is a real, unaltered photograph — not edited, not AI-generated, not an illustration.
- The image has no watermark and no copyright notice printed on it. (Screenshots are fine; crop out any surrounding text, captions, or logos.)
- The image is clear, in focus, and shows the clinically relevant feature.
- It is a single image — no collages or grids — and no personally identifiable information is the subject.
- The image is not a duplicate or a near-duplicate of one already submitted, and is not just a similar shot of the same case.
- You selected the correct domain from the five (Wound Triage, Skin Lesion Screening, Burn Classification, Rash Identification, Red-Eye Triage).
- The image genuinely belongs to that domain. (password is “readthis”)
- You added a short context note — a few words, one short sentence max — naming what the image shows (e.g. “This is melanoma”), and it is accurate.
- You saved the source URL of the image in case a reviewer asks.

Common reasons a task gets rejected

1. The image is blurry, low-resolution, or does not clearly show a clinically relevant feature.
2. The image has a watermark, a copyright notice, or a caption that gives away the answer.
3. The image is edited, AI-generated, or an illustration rather than a real photograph.
4. The image is a duplicate, a near-duplicate, or too similar to one already in the dataset.
5. The wrong domain was selected, or the image does not fit any of the five domains.
6. The context note is missing, too long, inaccurate, or describes what to do instead of naming the subject.
7. Personally identifiable information is in the image, or the image is a collage / multi-image grid.

When in doubt, picture the nurse who has to write a triage question and answer from your photo, and the model that will be graded on a safe vs. unsafe triage call. If your image is clear, on-domain, unique, and accurately labeled, you have built a good task.

7. Common Mistakes

These three issues account for most of the fixes we send back. Each is quick to avoid once you know to look for it, so check all three before you submit.

1. The context note is too long

This is the most common problem. The context note is a few words — one short sentence at most — naming what the image shows. We keep seeing notes that run on into severity, reasoning, and what the patient should do. All of that belongs in the prompt, not the context note. If your note is longer than one short sentence, cut it back to the subject.

✗ **Too long:** *"This is a deep partial-thickness burn on the palm that is extremely painful and will likely need referral to a burn center for proper treatment."*

✓ **Right:** *"This is a partial-thickness burn on the palm."*

2. Examples of poor images

Below is one example of each defect, with the rule it breaks:

1. Watermark



Problem: a stock-agency watermark is labeled at the bottom of the photo.

2. Visible copyright notice



Problem: a credit line is baked into the picture.

3. Blurry / out of focus



Problem: too pixelated to reason about the lesion or skin edges.

4. Collage / more than one image



Problem: a multi-image grid. Each task must be one single photo.

5. Answer-revealing text overlay



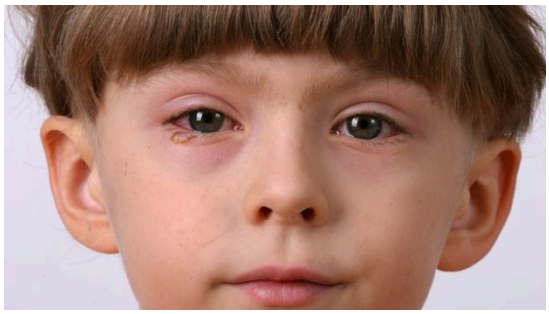
Problem: a caption naming the condition leaks the answer.

6. Illustration / AI-generated



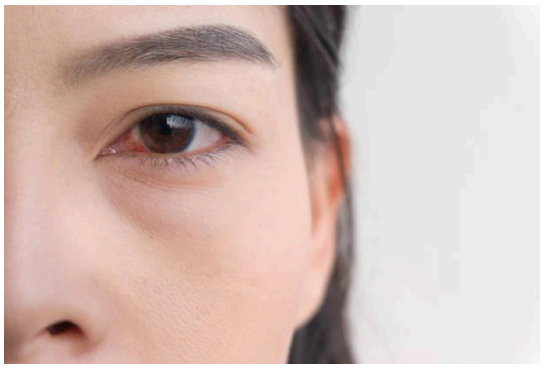
Problem: a drawing, not a real photograph.

7. Personally Identifiable Information



Problem: a full face is the subject; it should be cropped to the clinical area.

8. Staged, too-perfect stock photo



Problem: a clean studio shot that does not reflect a real presentation.